

Ms. Vernica Sadh

Panel : Meditest.in
378/18, Delhi-Jaipur Expressway, NH-8,
Rajiv Chowk, Sector 11, Gurugram,122001
9001290014
Referred By : self

Patient Id : 012410010251
Collected : 01 Oct 2024 05:48 PM
Analysed : 01 Oct 2024 07:04 PM
Reported : 01 Oct 2024 07:04 PM
Status : Final



7156319

2 Years/Female

DEPARTMENT OF HAEMATOLOGY

MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
Complete Blood Count (CBC) Extended			
Haemoglobin (Hb) (Whole Blood, Colorimetry)	11.9	gm/dL	11.0-14.0
TLC (Total Leucocyte Count) (Whole Blood, Flourescence Flowcytometry (SF Cube))	11920	/cumm	5000-15000
RBC Count (Whole Blood, Electrical Impedance)	4.53	10 ⁶ /uL	3.9-5.1
Packed Cell Volume (Whole Blood, Calculated)	35.9	%	30-38
MCV (Whole Blood, Impedance)	79.3	fL	72-84
MCH (Whole Blood, Calculated)	26.3	pg	25-29
MCHC (Whole Blood, Calculated)	33.1	g/dL	32-36
Platelet Count (Whole Blood, Flourescence Flowcytometry (SF Cube))	4.00	lakh/cumm	2.0-5.5
Differential Leucocyte Count			
Neutrophil (Whole Blood, Flourescence Flowcytometry (SF Cube))	26	%	35-45
Lymphocyte (Whole Blood, Flourescence Flowcytometry (SF Cube))	66	%	35-65
Eosinophils (Whole Blood, Flourescence Flowcytometry (SF Cube))	3	%	01-06
Monocytes (Whole Blood, Flourescence Flowcytometry (SF Cube))	5	%	0.1-8.0
Basophils (Whole Blood, Flourescence Flowcytometry (SF Cube))	0	%	0-1
Absolute Neutrophil Count (Whole Blood, Calculated)	3.1	x 10 ³ /uL	1.5-8.0
Absolute Lymphocyte Count (Whole Blood, Calculated)	7.87	x 10 ³ /uL	3.5-11.0
Absolute Eosinophil Count (Whole Blood, Calculated)	0.36	x 10 ³ cells/uL	0.1-1.0
Absolute Monocyte Count (Whole Blood, Calculated)	0.6		0.2-1.0
Absolute Basophil Count (Whole Blood, Calculated)	0	x 10 ³ cells/uL	0.00-0.00
Band Forms	0	%	

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Test Name	Value	Unit	Bio Ref.Interval
(Whole Blood, Microscopy)			
Metamyelocytes	0	%	
(Whole Blood, Microscopy)			
Myelocytes	0	%	
(Whole Blood, Microscopy)			
RDW-SD	40.6	fL	39.0-46.0
(Whole Blood, Calculated)			
RDW-CV	13.5	%	11.5-14.5
(Whole Blood, Calculated)			
MPV	11.0	fL	9.4-12.3
(Whole Blood, Calculated)			
PCT	0.44	%	0.10-0.28
(Whole Blood, Calculated)			
PDW	15.7	fL	9.0-17.0
(Whole Blood, Calculated)			
Menzter Index	17.51	Ratio	
(Whole Blood, Calculated)			
Erythrocyte Sedimentation Rate (ESR)	10	mm/h	0-10
(Whole Blood, Westergren)			

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HbA1c (Glycated Haemoglobin)

4.8

%

4.5-6.5

(HPLC Method)

REMARKS- *In vitro* quantitative determination of **HbA1c** in whole blood is utilized in long term monitoring of glycemia. The **HbA1c** level correlates with the mean glucose concentration prevailing in the course of the patient's recent history (approx - 6-8 weeks) and therefore provides much more reliable information for glycemia monitoring than do determinations of blood glucose or urinary glucose. It is recommended that the determination of **HbA1c** be performed at intervals of 4-6 weeks during Diabetes Mellitus therapy. Results of **HbA1c** should be assessed in conjunction with the patient's medical history, clinical examinations and other findings.

Mean Plasma Glucose

91.06

mg/dL

73 - 140

(Whole Blood, Calculated)

It is based on estimated Average Glucose (eAG) value calculated according to National Glycohemoglobin Standardization Program (NGSP) criteria.

The American Diabetes Association(ADA) recommendations are summarized as below:

Ref Range for HbA1c (In %):

Non diabetic Adults (Age >=18 years) < 5.7

At risk (Pre-Diabetic) : 5.7- 6.4

Diagnosing Diabetes: >= 6.5

HbA1c goals in treatment of diabetes:

Ages 0-6 years: 7.6% - 8.4%

Ages 6-12 years: <8%

Ages 13-19 years: <7.5%

Adults: <7%

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Blood Picture - Peripheral Smear Examination (P/S)

RBCs Normocytic Normochromic.

WBCs Total Leukocytes & Differential counts are within normal limits.

Platelets Platelets are Adequate in Smear.

Haemoparasites Not seen.

Immature cells Not seen.

Impression Normocytic Normochromic Blood Picture.

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DEPARTMENT OF BIOCHEMISTRY

MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
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Blood Glucose Fasting

71.0

mg/dL

60-100

(Flouride Plasma, Hexokinase)

Interpretation (In accordance with the American diabetes association guidelines):

- A fasting plasma glucose level below 100 mg/dL is considered normal.
- A fasting plasma glucose level between 100-126 mg/dL is considered as glucose intolerant or pre diabetic. A fasting and post-prandial blood sugar test (after consumption of 75 gm of glucose) is recommended for all such patients.
- A fasting plasma glucose level of above 126 mg/dL is highly suggestive of a diabetic state. A repeat fasting test is strongly recommended for all such patients. A fasting plasma glucose level in excess of 126 mg/dL on both the occasions is confirmatory of a diabetic state.

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DEPARTMENT OF BIOCHEMISTRY

MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
Liver Function Test (LFT) with GGT			
Bilirubin, Total (Serum, Dichlorophenyl Diazonium)	0.28	mg/dL	0.0-1.30
Bilirubin, Direct (Serum, Colorimetric, Diazo Dye)	0.10	mg/dL	0.00-0.50
Bilirubin, Indirect (Serum, Calculated)	0.18	mg/dL	0.10-1.00
SGOT (AST), Serum (Serum, NADH w/o P-5-P)	43.00	U/L	<31
SGPT (ALT), Serum (Serum, NADH (without P-5-P))	19.00	IU/L	<34
ALP (Alkaline Phosphatase), Serum (Serum, Para-nitrophenyl Phosphate)	272.0	U/L	54-369
Protein, Total (Biuret)	7.07	g/dL	6.0-8.0
Albumin (Serum, Bromocresol green)	4.59	g/dL	3.80-5.40
Globulin	2.48	g/dL	2.00-3.50
A/G Ratio	1.85		
GGT (Gamma Glutamyl Transferase) (Serum, IFCC)	10	U/l	<38
ALT/AST Ratio (Serum, Calculated)	0.44	Ratio	

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MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
Kidney Function Test (KFT/RFT)			
Blood Urea Nitrogen (BUN) (Serum, Urease)	15.40	mg/dL	5.0-18.0
Urea, Blood (Serum, Calculated)	32.96	mg/dL	11.00-36.00
Creatinine (Serum, Kinetic Alkaline Picrate)	0.52	mg/dL	0.5-1.11
Uric Acid, Serum (Serum, Uricase)	3.20	mg/dL	2.6-6.0
Sodium, Serum (Serum, Ion Selective Electrode)	138.0	mmol/L	138.0-145.0
Potassium, Serum (Serum, Ion Selective Electrode)	4.30	mmol/L	3.4-4.7
Chloride (Ion Selective Electrode)	105.0	mmol/L	98.0-109.0
Calcium, Serum (Serum, Arsenazo III)	9.70	mg/dL	8.6-10.2
Phosphorous (Serum, Phosphomolybdate)	6.40	mg/dl	4.00-7.00
BUN / Creatinine Ratio (Serum, Calculated)	29.62		
Urea / Creatinine Ratio (Serum, Calculated)	63.38		

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MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
Lipid Profile			
Total Cholesterol (Serum, CHOD-PAP)	134.0	mg/dL	<170
Triglyceride (Serum, Glycerol Phosphate Oxidase)	117.0	mg/dL	<150
HDL Cholesterol (Serum, ASD(accelerator selective detergent))	50.0	mg/dL	40-60
VLDL Cholesterol (Calculated)	23.4	mg/dL	4.7-21.1
LDL Cholesterol (Calculated)	60.6	mg/dL	0-100
Non-HDL Cholesterol (Calculated)	84	mg/dL	<130
LDL / HDL Cholesterol Ratio	1.21	%	2.5-3.5
Total / HDL Cholesterol Ratio (Serum, Calculated)	2.68	Ratio	3.00-5.00

Comment - Lipid profile checks cholesterol levels, comprising of parameters total cholesterol, LDL cholesterol, HDL cholesterol and triglycerides. The results of the lipid profile as per the AHA guidelines mentioned below.

Guidelines from The American Heart Association (AHA)

Total Cholesterol (mg/dL)		HDL Cholesterol (mg/dL)	
<200	Best	<40 (men); <50 (women)	Poor
200-239	Borderline high	50-59	Better
>=240	High	>=60	Best

Triglyceride (mg/dL)		LDL Cholesterol (mg/dL)		Non-HDL (mg/dL)	
<150	Best	<70	Best for people with heart disease	<130	Best
150-199	Borderline high	<100	Best for people at risk of heart disease.	130-159	Near Ideal
200-499	High	100-129	Near ideal	160-189	Borderline high
>=500	Very high	130-159	Borderline high	190-219	High
		160-189	High	>=220	Very high
		>=190	Very high		

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DEPARTMENT OF IMMUNOASSAY

MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
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Ferritin

FERRITIN SERUM
(Serum, CMIA)

10.09

ng/ml

5.0-204.0

Comment:

Ferritin is the major iron storage compound and a readily available source & reserve of iron for metabolic requirements.

Clinical utility:

- Ferritin estimation is useful in the diagnosis of iron deficiency anemia and iron overload.
- Increased levels seen in hemochromatosis, frequent blood transfusions with packed RBCs and alcoholic liver disease.
- Decreased levels seen in heavy menstrual bleeding, poor absorption of iron, iron deficiency anaemia and long-term GI bleed.

Note:

Ferritin is an acute phase reactant and thus may be increased with inflammation, chronic infection, liver disease, autoimmune disorders and some type of cancers. Ferritin is not used to detect or monitor these conditions.

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MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
Vitamin D, 25 Hydroxy (Serum, CMIA)	14.50	ng/mL	30-100

Interpretation :

- Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.
- Testing for 25(OH)vitamin D is recommended as it is the best indicator of vitamin D nutritional status as obtained from sunlight exposure & dietary intake. For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.
- During monitoring of oral vitamin D therapy- suggested testing of serum 25(OH)vitamin D is after 12 weeks or 3 mths of treatment. However, the required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal(especially winter) & individual variability depending on age, body fat, sun exposure, physical activity, genetic factors(especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism Vitamin D toxicity is known but very rare.kindly correlate clinically, repeat with fresh sample if indicated.
- For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D and serum PTH.An inverse relationship exists between PTH and 25(OH)D levels, Parathyroid hormone levels start to rise at 25(OH)D levels below 31 ng/mL & usually decrease after the correction of vitamin D insufficiency.Thus, restoration of PTH and 25 (OH)D levels to normalcy after adequate vitamin D replacement therapy is a useful monitoring strategy.

Reference Range in ng/mL	Level
<10	Deficiency (Severe)
10-19.99	Deficiency (Mild to Moderate)
20-29.99	Insufficiency
30-99.99	Optimum Level
>100	Toxicity Possible

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DEPARTMENT OF IMMUNOASSAY

MediTest Obesity Screening Package

Test Name	Value	Unit	Bio Ref.Interval
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TFT (Thyroid Function Test) Total

Triiodothyronine, Total (T3) (Serum, CMIA)	147.00	ng/dL	105-269
Thyroxine, Total (T4) (Serum, CMIA)	7.04	ug/dL	7.3-15.0
TSH Ultra Sensitive (Serum, CMIA)	1.89	μIU/ml	0.7-6.4

Comment

T₃ or 3,5,3,5-triiodothyronine is a hormone synthesized and secreted from the thyroid gland, and formed by peripheral deiodination of thyroxine (T₄). The determination of it in serum is essential in assessing thyroid functions. T₃ is secreted by thyroid glands and circulates in the blood stream; mostly bound to the plasma protein, thyroxine binding globulin (TBG) and prealbumin and albumin.

T₄ or Thyroxine or 3,5,3,5-tetraiodothyronine is a hormone synthesized and secreted by the thyroid gland and plays an important role in regulating metabolism. In the peripheral tissues it act as a prohormone which is further metabolized to another most active thyroid hormone, triiodothyronine (T₃) and other inactive metabolites such as reverse T₃.

TSH or Thyroid-stimulating hormone is a hormone synthesized and secreted by Pituitary gland. TSH is glycoprotein with two non-covalently bound alpha and beta subunits. The beta subunit of TSH is unique, which results in the specific biochemical and immunological properties of this hormone. The ability to quantitate circulating levels of TSH is important in evaluating thyroid function. It is especially useful in the differential diagnosis of primary (thyroid) from secondary (pituitary) and tertiary (hypothalamus) hypothyroidism.

Reference Ranges for T₃, T₄ and TSH (Age Wise and During Pregnancy):

Age	Total T ₄ (μg/dl)
0 - 6 days	11.8 - 22.6
1w - 2w	9.9 - 16.6
1 - 4 months	7.2 - 14.4
4 months - 1 years	7.8 - 16.5
1 - 5 years	7.3 - 15.0
5 - 10 years	6.4 - 13.3
10 - 15 years	5.6 - 11.7
15 - 60 years (Male)	4.6 - 10.5
15 - 60 years (Female)	5.5 - 11.0
>60 years	5.0 - 10.7

Age	Total T ₃ (ng/dl)
0 - 3 days	100 - 740
4 days - 11 months	105 - 245
4 - 5 years	105 - 269
6 - 10 years	94 - 241
11 - 15 years	82 - 213
16 - 20 years	80 - 210
20 - 50 years	70 - 204
50 - 90 years	40 - 181

Age	TSH (μIU/ml)
0 - 4 days	1 - 39.0
5 days - 90 days	1.7 - 9.1
91 days - 20 years	0.7 - 6.4
21 - 54 years	0.35 - 5.5
>54 years	0.5 - 8.9

Pregnancy Trimester	TSH (μIU/ml)
First Trimester	0.10 - 2.5
Second Trimester	0.20 - 3.0
Third Trimester	0.30 - 3.0

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Test Name	Value	Unit	Bio Ref.Interval
Tissue Transglutaminase Antibody IgA (Serum, ELISA)	5.60	U/mL	Negative < 12 Borderline 12-18 Positive > 18

Interpretation

Tissue Transglutaminase (tTG) test was performed by a solid phase enzyme immunoassay for the quantitative and qualitative detection of antibodies against neo-epitopes of tissue transglutaminase (tTG) in human serum.

Clinical Significance

Gluten-sensitive enteropathy or celiac disease is characterized by atrophy of the small intestinal villi leading to a so-called flat mucosa. It is caused by a pathological intolerance to gliadin, the alcohol-soluble fraction of gluten in wheat, rye and barley. As celiac disease is caused by the uptake of gluten, consequently a gluten-free diet cures the disease completely and thus has to be maintained for life-time.

Diagnosis of celiac disease is made by small intestinal biopsy (demonstrating flat mucosa) supported by serological markers. Antibodies against gliadin and anti-endomysial antibodies (EMA) are of major significance.

The diagnostic specificity of tissue transglutaminase (tTG) antibodies for Celiac disease is 95 -100%. The diagnostic sensitivity of tissue transglutaminase (tTG) antibodies for Celiac is 98 - 100%.

*** End Of Report ***



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